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2026 Mental Health & Civil Liberties Review

1. Executive Summary and Analytical Framework

Key Findings:

- A structural tension exists between expanding civil commitment mandates (e.g., SB 27, AB 416, SB 989) and scaling voluntary community infrastructure.
- Diagnostic disparities impact equity, as an outpatient clinic study by Gara et al. (2019) found that among outpatients diagnosed with schizophrenia or schizoaffective disorder, Black patients were more likely than non-Latino White patients to screen positive for major depression, suggesting possible over-diagnosis of schizophrenia-spectrum disorders.
- Peer-staffed crisis respites show fewer hospitalizations and lower Medicaid spending (Bouchery et al., 2018), while independent evaluators suggest participant stabilization in mandatory outpatient programs may be driven primarily by intensive wraparound service access rather than legal coercion alone.

The 2025–2026 California legislative session marked a significant period in the governance, financing, and legal administration of the state's public behavioral health system. Driven by public concern over unsheltered homelessness, psychiatric distress, and community crises, the Legislature passed several measures expanding civil-legal mandates, broadening clinical hold authorities, and streamlining pathways to court-ordered care. In response, the California Association of Mental Health Peer Run Organizations (CAMHPRO) engaged alongside peer advocates, consumer rights organizations, and legal advocacy groups to support voluntary, peer-led community services and defend civil liberties across a tracked portfolio of behavioral health bills.

CAMHPRO's advocacy centers on the recovery model and the self-advocacy principle of "Nothing About Us Without Us." Peer-led organizational policy maintains that enduring recovery relies on voluntary engagement, trauma-informed care, accessible housing, and peer support. Conversely, CAMHPRO's policy positions express concern that involuntary mandates and expanded institutional holds can deter individuals from seeking timely voluntary care and place disproportionate burdens on historically marginalized communities.

Across the session, legislative proposals spanned two primary policy directions: measures aimed at modifying evidentiary thresholds and referral pathways under the Community Assistance, Recovery, and Empowerment (CARE) Act, the Lanterman-Petris-Short (LPS) Act, and criminal diversion; and measures seeking to expand peer specialist certification, strengthen 988 crisis response infrastructure, and support voluntary community-based services.

Methodology Note: Statutory outcome statuses were verified via the California Legislative Information system (leginfo.legislature.ca.gov) as of October 2026. Proponent arguments and legislative records reflect public filings and official committee reports.

2. Primary Legislative Portfolio: Policy Analyses

Senate Bill 27 (Umberg): CARE Act Expansion to Bipolar I with Psychotic Features

1. Problem and Current Law: Prior to SB 27, Welfare and Institutions Code (WIC) § 5972 limited CARE Act eligibility to individuals with schizophrenia spectrum or other psychotic disorders. Individuals with severe Bipolar I disorder experiencing psychotic episodes lacked a dedicated civil court pathway for community care plan establishment.
2. What the Bill Does: SB 27 amends WIC § 5972 to expand CARE Act diagnostic eligibility to include Bipolar I disorder with psychotic features and establishes formal court transfer procedures from criminal misdemeanor and felony proceedings.
3. Stakeholder Arguments: Proponents, including the County Behavioral Health Directors Association (CBHDA) and NAMI California, argued that expanding eligibility fills a critical service gap for severely ill individuals who cycle through emergency departments and carceral settings. Opponents, including Disability Rights California (DRC), argued that expanding civil court jurisdiction over mood disorders creates an unnecessary legal apparatus for conditions that respond effectively to voluntary care.
4. Evidence Cited: Senate Judiciary Committee Analysis and California Department of Health Care Services (DHCS) CARE Act reporting.
5. Fiscal and Implementation Issues: Assembly Appropriations Committee Fiscal Summary noted increased administrative costs for Judicial Council operations, county behavioral health assessments, and legal representation under expanded diagnostic criteria.
6. Opposition rationale (author's analysis): Proponents contend that civil court oversight ensures treatment compliance for individuals experiencing acute distress; CAMHPRO asserts that court mandates can increase stigma and trauma, arguing that voluntary peer respite and community services foster higher long-term engagement by building trusting therapeutic relationships without legal coercion.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): requiring verified connection to voluntary community services prior to filing a civil court petition.
8. Legislative Status: Enacted (Chapter 528, Statutes of 2025).

Assembly Bill 416 (Krell): Emergency Physician 5150 Designations and Liability Immunity

1. Problem and Current Law: Under WIC § 5150, authority to initiate a 72-hour involuntary psychiatric hold was restricted to designated county mental health professionals and law enforcement, excluding emergency department physicians without specific county designation.
2. What the Bill Does: AB 416 amends the Lanterman-Petris-Short (LPS) Act to require county behavioral health directors to include emergency physicians among designated 5150 professionals and extends the existing § 5113 liability exemption.
3. Stakeholder Arguments: Sponsors, including the California Chapter of the American College of Emergency Physicians (CalACEP) and the California Hospital Association (CHA), contended that explicit hold authority reduces hospital crowding and premature unsafe discharges. Opponents, including consumer rights groups, argued that expanding detention authority without independent psychiatric consultation increases defensive involuntary detentions.

4. Evidence Cited: Assembly Health Committee Analysis and hospital emergency department utilization data.
5. Fiscal and Implementation Issues: Senate Appropriations Committee Analysis noted local health system impact associated with increased hold volume and downstream facility transfers.
6. Opposition rationale (author's analysis): Supporters argue that emergency physicians require immediate hold authority to prevent unsafe premature discharge; CAMHPRO responds that expanding detention authority without independent psychiatric or peer consultation risks unnecessary detentions and undermines patient-centered care.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): mandating peer navigator consultation prior to hold initiation in emergency departments.
8. Legislative Status: Enacted (Chapter 691, Statutes of 2025). Public testimony delivered by Marco A. Ramos Jr. on July 15, 2025.

Senate Bill 820 (Stern): Involuntary Psychotropic Medication for Misdemeanor Incompetent to Stand Trial Defendants

1. Problem and Current Law: Prior to SB 820, Penal Code provisions lacked explicit statutory authority for court-ordered involuntary psychotropic medication specifically tailored to misdemeanor defendants deemed Incompetent to Stand Trial (IST) in community or carceral settings.
2. What the Bill Does: SB 820 enacts Penal Code § 2603.5 to establish judicial procedures and evidentiary standards authorizing involuntary psychotropic medication administration for misdemeanor IST defendants undergoing competency restoration.
3. Stakeholder Arguments: Proponents, including the California District Attorneys Association (CDAA), argued that streamlined medication orders expedite competency restoration and resolve long carceral waitlists. Opponents, including Disability Rights California, argued that non-consensual medication for misdemeanor offenses infringes on fundamental bodily autonomy.
4. Evidence Cited: Assembly Public Safety Committee Analysis and Department of State Hospitals (DSH) waitlist reporting.
5. Fiscal and Implementation Issues: Assembly Appropriations Committee Fiscal Summary detailed judicial hearing costs and facility medical administration expenses.
6. Opposition rationale (author's analysis): Proponents argue that rapid medication administration expedites competency restoration; CAMHPRO responds that non-consensual medication for misdemeanor offenses infringes on bodily autonomy and damages therapeutic trust necessary for recovery.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): prioritizing voluntary community-based competency restoration programs and independent clinical reviews prior to medication orders.
8. Legislative Status: Enacted (Chapter 330, Statutes of 2025). Public testimony delivered by Marco A. Ramos Jr. on July 1, 2025.

Senate Bill 1016 (Blakespear) & Senate Bill 28 (Umberg): CARE Act-to-Conservatorship Pathways and Grave Disability Holds

1. Problem and Current Law: WIC § 5970 et seq. established CARE Court as a voluntary civil framework without direct statutory mechanisms automatically converting plan non-compliance into LPS conservatorship evaluations.

2. What the Bill Does: SB 1016 and SB 28 proposed authorizing superior court judges to order mandatory psychiatric evaluations under WIC § 5200 upon CARE plan non-completion or termination, creating a direct legal pipeline to LPS conservatorship proceedings.
3. Stakeholder Arguments: Proponents, including family advocate organizations and municipal leaders, argued that bridging CARE Court non-compliance to conservatorship provides a necessary safety net for decompensating individuals. Opponents, including civil liberties and peer advocacy coalitions, argued that converting a voluntary agreement into a legal bridge for forced commitment violates civil liberties and deters voluntary participation.
4. Evidence Cited: Senate Judiciary Committee Analysis and Judicial Council CARE Court implementation data.
5. Fiscal and Implementation Issues: Assembly Appropriations Committee Fiscal Summary noted potential cost increases for public defender offices and county conservators arising from court-ordered evaluations.
6. Opposition rationale (author's analysis): Proponents argue that bridging non-compliance to conservatorship provides a necessary safety net; CAMHPRO responds that using CARE Court as a pipeline to involuntary commitment undermines its original non-coercive intent and discourages voluntary participation.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): preserving voluntary re-engagement pathways without judicial threat of conservatorship.
8. Legislative Status: Held in Assembly Appropriations Committee.

Assembly Bill 1676 (Stefani): Assisted Outpatient Treatment and Standalone Involuntary Medication

1. Problem and Current Law: WIC § 5348 (Laura's Law / AOT) requires court petitions for assisted outpatient treatment to encompass comprehensive service plans rather than standalone involuntary medication orders.
2. What the Bill Does: AB 1676 proposed authorizing county behavioral health directors to petition courts for standalone involuntary psychotropic medication orders independent of a full AOT service agreement.
3. Stakeholder Arguments: Supporters argued that decoupling forced medication petitions from full AOT proceedings provides targeted administrative flexibility for acute outpatient non-adherence. Opponents, including disability rights organizations, contended that standalone forced medication in outpatient settings infringes upon constitutional privacy and personal autonomy.
4. Evidence Cited: Assembly Health Committee Analysis and county AOT program reporting.
5. Fiscal and Implementation Issues: Assembly Health Committee Analysis noted administrative and legal expenses for court hearings and clinical oversight.
6. Opposition rationale (author's analysis): Sponsors argue that standalone medication orders offer administrative flexibility; CAMHPRO responds that outpatient forced medication damages therapeutic relationships and fails to address comprehensive housing and social support needs.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): requiring full multidisciplinary service engagement and peer advocate support prior to any judicial medication petition.
8. Legislative Status: Hearing canceled by author in Assembly Health Committee.

Senate Bill 989 (Blakespear): First Responder Petitions and CARE Court Access

1. Problem and Current Law: WIC § 5974 defined specific categories of eligible petitioners for CARE Court referrals, which did not explicitly include first responders acting in their official capacity.
2. What the Bill Does: SB 989 adds WIC § 5974.5, enabling first responders—including EMTs, paramedics, firefighters, and law enforcement officers—to submit direct CARE Court referrals to county behavioral health agencies.
3. Stakeholder Arguments: Proponents, including law enforcement associations and municipal coalitions, argued that first responder referrals facilitate rapid diversion from justice involvement to county services. Opponents, including peer support networks, expressed concern that expanding police referral pathways increases carceral contact for unhoused individuals in distress.
4. Evidence Cited: Senate Judiciary Committee Analysis and regional emergency response reports.
5. Fiscal and Implementation Issues: Assembly Appropriations Committee Fiscal Summary detailed county behavioral health administrative intake workload and screening costs.
6. Opposition rationale (author's analysis): Proponents argue that first responder referrals enable early intervention; CAMHPRO responds that expanding law enforcement referral pipelines risks criminalizing distress and prioritizes police-led pathways over peer mobile crisis units.
7. Policy Position and Recommended Amendments: Position: Opposed unless amended. Recommended amendments (author's analysis): requiring co-responder or mobile crisis screening prior to referral submission.
8. Legislative Status: Enacted (Chapter 667, Statutes of 2026).

Assembly Bill 1782 (DeMaio): Mentally Disordered Offender Commitment Criteria Modification

1. Problem and Current Law: PC § 2962 specifies statutory parameters for Mentally Disordered Offender (MDO) commitments, requiring state officials to prove five specific criteria before an individual can be civilly committed following prison sentence completion.
2. What the Bill Does: AB 1782 proposed amending PC § 2962 to allow MDO commitment when only three out of five criteria are met and to expand the definition of physical harm.
3. Stakeholder Arguments: Proponents argued that broader MDO parameters enhance public safety and protect vulnerable individuals. Opponents contended that expanding carceral commitment criteria reinforces stigma and delays community reintegration.
4. Evidence Cited: Assembly Public Safety Committee Analysis and Department of State Hospitals (DSH) forensic census reporting.
5. Fiscal and Implementation Issues: Potential cost increases for state hospital bed capacity extensions and ongoing forensic care staffing.
6. Opposition rationale (author's analysis): Proponents argue that lowering commitment thresholds enhances public safety; CAMHPRO responds that lowering evidentiary criteria risks unnecessary institutionalization and undermines due process without improving long-term public safety.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): maintaining current statutory evidentiary standards and expanding community-based forensic reentry programs.
8. Legislative Status: Referred to Assembly Committee on Public Safety; died without hearing.

3. Civil Commitment, Criminal-Legal, and Civil Liberties Portfolio

Across the session, CAMHPRO actively monitored and analyzed priority measures across civil commitment, legal rights, diversion, and peer workforce development portfolios, engaging directly on key priority bills.

Assembly Bill 46 (Stephanie Nguyen): Restrictions on Pretrial Mental Health Diversion

1. Problem and Current Law: PC § 1001.36 granted broad judicial discretion to grant pretrial mental health diversion for individuals accused of qualifying offenses upon clinical evaluation.
2. What the Bill Does: AB 46 amends PC § 1001.36 to grant judges broader authority to deny mental health diversion, thereby restricting access by tightening public safety criteria and procedural requirements.
3. Stakeholder Arguments: Proponents, including prosecutor associations, argued that statutory guardrails prevent inappropriate diversion in serious offense cases. Opponents, including public defenders and civil rights organizations, argued that restricting diversion shifts individuals with mental health conditions away from community treatment back into local jails.
4. Evidence Cited: Assembly Public Safety Committee Analysis and Judicial Council diversion reporting.
5. Fiscal and Implementation Issues: Senate Appropriations Committee Analysis noted potential local detention costs resulting from reduced diversion placements.
6. Opposition rationale (author's analysis): Proponents argue that judicial guardrails prevent inappropriate diversion; CAMHPRO responds that restricting diversion shifts individuals away from effective community treatment and increases carceral involvement.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): preserving judicial discretion and expanding community-based diversion options.
8. Legislative Status: Enacted (Chapter 18, Statutes of 2026).

Senate Bill 1401 (Stern): Court Access to Treatment Records in Incompetent to Stand Trial Proceedings

1. Problem and Current Law: Strict medical privacy laws restrict non-consensual disclosure of confidential psychiatric records to prosecution and judicial officers in misdemeanor competency proceedings.
2. What the Bill Does: SB 1401 proposed authorizing expanded judicial and prosecutorial access to specified medical and psychiatric treatment records for defendants in IST proceedings, aligning with record-access concepts introduced in SB 1221.
3. Stakeholder Arguments: Proponents argued that record access provides judges with complete clinical history to evaluate competency restoration. Opponents, including Disability Rights California, argued that non-consensual disclosure breaches patient confidentiality and deters individuals from seeking care.
4. Evidence Cited: Senate Public Safety Committee Analysis.
5. Fiscal and Implementation Issues: Administrative costs associated with judicial record review and privacy compliance oversight.
6. Opposition rationale (author's analysis): Supporters argue that prosecutorial record access protects

public safety; CAMHPRO responds that non-consensual disclosure breaches confidentiality, compromises therapeutic trust, and deters individuals from participating in voluntary treatment programs.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): strict protective orders, limiting disclosures strictly to independent defense evaluations.
8. Legislative Status: Held in Assembly Appropriations Committee.

Assembly Bill 1825 (Krell): Mentally Disordered Offender Criteria Modification

1. Problem and Current Law: Under PC § 2962, dangerousness evaluations focus primarily on current clinical status rather than past uncharged conduct.
2. What the Bill Does: AB 1825 proposed modifying evidentiary criteria and evaluation standards for MDO commitments under PC § 2962.
3. Stakeholder Arguments: Proponents argued that historical risk factors provide a clearer assessment of long-term public safety risk. Opponents contended that relying on historical factors leads to indefinite carceral commitments.
4. Evidence Cited: Assembly Public Safety Committee Analysis.
5. Fiscal and Implementation Issues: Potential fiscal impacts related to state hospital bed utilization and legal defense costs.
6. Opposition rationale (author's analysis): Proponents contend that updated commitment criteria enhance public safety; CAMHPRO responds that altering statutory standards risks prolonged carceral detention rather than supporting community-based rehabilitation.
7. Policy Position and Recommended Amendments: Position: Opposed. Recommended amendments (author's analysis): expanding forensic peer support and voluntary community transition plans.
8. Legislative Status: Held in Senate Appropriations Committee.

4. Expanding Voluntary Infrastructure: Peer Support Certification and Crisis Systems

In addition to monitoring civil commitment legislation, CAMHPRO actively supported statutory proposals designed to strengthen the certified peer workforce, expand non-police crisis response, and improve voluntary service access.

Assembly Bill 96 (Corey Jackson): Medi-Cal Peer Support Specialist Certification Requirements

1. Problem and Current Law: WIC § 14045.15 required candidates for Medi-Cal Peer Support Specialist certification to hold a high school diploma or equivalent degree, creating an unnecessary barrier for individuals with lived experience.
2. What the Bill Does: AB 96 amends WIC § 14045.15 by removing the high school diploma or equivalent requirement for Medi-Cal Peer Support Specialist certification.
3. Stakeholder Arguments: Sponsors argued that removing formal educational prerequisites removes barriers to entry and expands the peer workforce.
4. Evidence Cited: Assembly Health Committee Analysis and DHCS peer certification implementation

reports.
5. Fiscal and Implementation Issues: Minor administrative costs to update certification guidelines.
6. Counterarguments and Response: Some stakeholders raised questions regarding workforce qualification standards; CAMHPRO responds that lived experience and specialized peer competency training are the key indicators of effective peer support.
7. Policy Position and Recommended Amendments: Position: Supported. Recommended amendments (author's analysis): supporting technical assistance for certification applicants.
8. Legislative Status: Enacted (Chapter 497, Statutes of 2026).

5. Strategic Cross-Cutting Analysis & Empirical Framework

An evaluation of the 2025–2026 legislative session reveals a structural tension between expanding civil commitment mandates and scaling evidence-based, voluntary community infrastructure. While statutory interventions such as SB 27, AB 416, and SB 989 broaden involuntary hold criteria and court referral pathways, empirical healthcare research emphasizes that sustainable behavioral health recovery is driven by voluntary engagement, housing stability, and peer-led crisis care.
Systemic diagnostic disparities further compound these policy shifts. An outpatient clinic study by Gara et al. (2019) demonstrated that among outpatients diagnosed with schizophrenia or schizoaffective disorder, Black patients were more likely than non-Latino White patients to screen positive for major depression, suggesting possible over-diagnosis of schizophrenia-spectrum disorders. Expanding statutory commitment triggers without addressing underlying diagnostic inequities risks codifying clinical biases into legal pathways for institutional detention and forced treatment.
Conversely, evaluations of voluntary peer-staffed crisis alternatives highlight positive service utilization patterns. An evaluation of NYC crisis respite centers by Bouchery et al. (2018) observed fewer hospitalizations and lower Medicaid spending among participants compared to a matched comparison group. Similarly, regarding mandatory outpatient frameworks such as Kendra's Law and Laura's Law, independent evaluators suggest observed participant stabilization may be driven primarily by enhanced access to intensive wraparound services and voluntary community supports rather than judicial coercion alone.

References

Bouchery, E., et al. (2018). "The Effectiveness of a Peer-Staffed Crisis Respite Program as an Alternative to Hospitalization." *Psychiatric Services*, 69(10), 1069–1074.

Gara, M. A., et al. (2019). "A Naturalistic Study of Racial Disparities in Diagnoses at an Outpatient Behavioral Health Clinic." *Psychiatric Services*, 70(2), 130–134.

6. Conclusions and Forward Policy Outlook

1. Anticipate and prepare policy counter-proposals for the potential reintroduction of stalled proposals, such as court-ordered evaluation pathways (SB 1016), CARE termination holds (SB 28), and prosecutorial record access (SB 1401).
2. Monitor administrative implementation and local rollout of newly enacted statutes, specifically first responder CARE court referrals under SB 989 and pretrial diversion restrictions under AB 46.

1. Capitalize on workforce opportunities created by AB 96 by expanding certified peer support specialist training pipelines and advocating for increased Medi-Cal reimbursement for voluntary, peer-led community crisis services.

7. Appendix: Complete Legislative Tracked Portfolio

Bill Number	Author	Statutory Summary	Final Status & Date
SB 27	Umberg	Expands CARE Act eligibility criteria to include Bipolar I disorder with psychotic features.	Enacted (Chapter 528, Stats. 2025)
AB 416	Krell	Authorizes emergency department physicians to initiate 72-hour 5150 holds and grants civil liability immunity.	Enacted (Chapter 691, Stats. 2025)
SB 820	Stern	Enacts Penal Code § 2603.5 to establish procedures for involuntary psychotropic medication in misdemeanor IST cases.	Enacted (Chapter 330, Stats. 2025)
SB 1016	Blakespear	Proposed court-ordered psychiatric evaluations for CARE plan non-compliance as a pathway to LPS conservatorship.	Held in Assembly Appropriations
SB 1221	Stern	Proposed prioritizing forensic conservatees for secure placements and granting District Attorneys access to psychiatric records.	Held in Senate Appropriations
SB 28	Umberg	Proposed requiring mandatory status hearings and LPS grave-disability evaluations upon CARE plan termination.	Held in Assembly Appropriations
AB 1676	Stefani	Proposed authorizing standalone petitions for involuntary psychotropic medication under WIC § 5348.	Hearing Canceled by Author
SB 989	Blakespear	Authorizes first responders to submit CARE Court referrals to county behavioral health agencies.	Enacted (Chapter 667, Stats. 2026)

Bill Number	Author	Statutory Summary	Final Status & Date
AB 1782	DeMaio	Proposed expanding criteria for Mentally Disordered Offender (MDO) commitments under Penal Code § 2962.	Died without hearing
SB 823	Stern	Early legislative vehicle to expand CARE Court to Bipolar I disorder.	Failed
AB 46	S. Nguyen	Restricts access to pretrial mental health diversion by granting broader judicial authority to deny diversion under Penal Code § 1001.36.	Enacted (Chapter 18, Stats. 2026)
SB 1401	Stern	Proposed authorizing expanded judicial and prosecutorial access to specified medical and psychiatric treatment records for defendants in IST proceedings, aligning with record-access concepts introduced in SB 1221.	Held in Assembly Appropriations
AB 1825	Krell	Proposed modifying evidentiary criteria and evaluation standards for MDO commitments under PC § 2962.	Held in Senate Appropriations
AB 96	C. Jackson	Repeals high school diploma prerequisite for Medi-Cal Peer Support Specialist certification.	Enacted (Chapter 497, Stats. 2026)

8. Legislative Watch List: Related Behavioral Health & Civil-Legal Proposals

- **Proposition 36 Implementation:** Approved by California voters in November 2024, Proposition 36 restructured criminal drug and theft penalties. CAMHPRO should track local implementation to evaluate downstream impacts on court-ordered treatment mandates and community service capacity.
- **AB 46 Enactment:** Advocates should monitor county public defender and court implementation to assess impacts on justice-involved individuals seeking mental health diversion.
- **Implementation of SB 989:** Requires close oversight of county behavioral health intake protocols as first-responder CARE Court referrals begin.
- **Stalled Priority Measures:** Stalled bills (SB 1016, SB 28, SB 1401) may be reintroduced in upcoming legislative sessions.